

Risk Summary

Preeclampsia

Medium Score: 0.6395082154771051

Model Interpretation: Risk-based triage only (not diagnosis).

Macrosomia

Medium Score: 0.5943816940585895

Model Interpretation: Risk-based coaching support.

Summary: The patient, at 32 weeks gestation, has a medium risk for preeclampsia and a medium risk for macrosomia. Her blood pressure is elevated (142/79 mmHg) with proteinuria (dipstick 1). No acute preeclampsia symptoms are reported. Glucose levels are currently within target, but pre-pregnancy BMI is high.

Patient Snapshot (Recent Inputs)

Parameter	Value	Notes
Blood Pressure	142/79 mmHg	Hypertensive threshold ≥140/90, severe ≥160/110.
Proteinuria (dipstick)	1	≥1+ supports PE suspicion in context of HTN.
Symptoms	Headache=0, Vision=0, RUQ Pain=0, Swelling=0	Severe features increase escalation priority.
Glucose	Fasting=99 mg/dL 1hr PP=126 mg/dL	Targets typically fasting <95, 1hr <140 (pregnancy goals).
HbA1c	5.6	Higher HbA1c indicates sustained glycemic elevation.
Maternal Context	Age=38 BMI=42.4 GDM=0	Risk modifiers for fetal overgrowth + metabolic risk.
Activity / Sleep	Post-meal walk=29 min Sleep=6.1 hrs	Lifestyle factors influence glycemic response.
Food Log	paneer salad	Used for dietary correlation patterns.

Reason for Alert

Action Requested: ALERT_DOCTOR

Clinical Rationale: ALERT: Patient TEST_MED_MED, 32 weeks gestation, has a medium preeclampsia risk tier (score: 0.6395) and a medium macrosomia risk tier (score: 0.5944). Key vitals include systolic BP 142 mmHg, diastolic BP 79 mmHg, and proteinuria dipstick 1. No severe headache, vision changes, upper abdominal pain, or swelling of face/hands reported. Given the elevated BP and proteinuria, further clinical evaluation for preeclampsia is recommended as per ACOG guidelines on hypertensive disorders of pregnancy. High pre-pregnancy BMI (42.4) noted, contributing to macrosomia risk despite current glucose levels (fasting glucose: 99 mg/dL, postprandial 1hr glucose: 126 mg/dL, HbA1c: 5.6%) being within target and no GDM diagnosis.

Suggested Next Steps

- Review trend: BP, symptoms, proteinuria, glucose readings
- Consider confirmatory tests (protein/creatinine ratio, LFTs, platelets) if clinically indicated
- Consider GDM diet review or insulin adjustment if glucose targets exceeded
- Provide patient education + return precautions

⚠ This report is generated by an AI decision-support system and does not replace clinical judgment. Always follow institutional protocols.